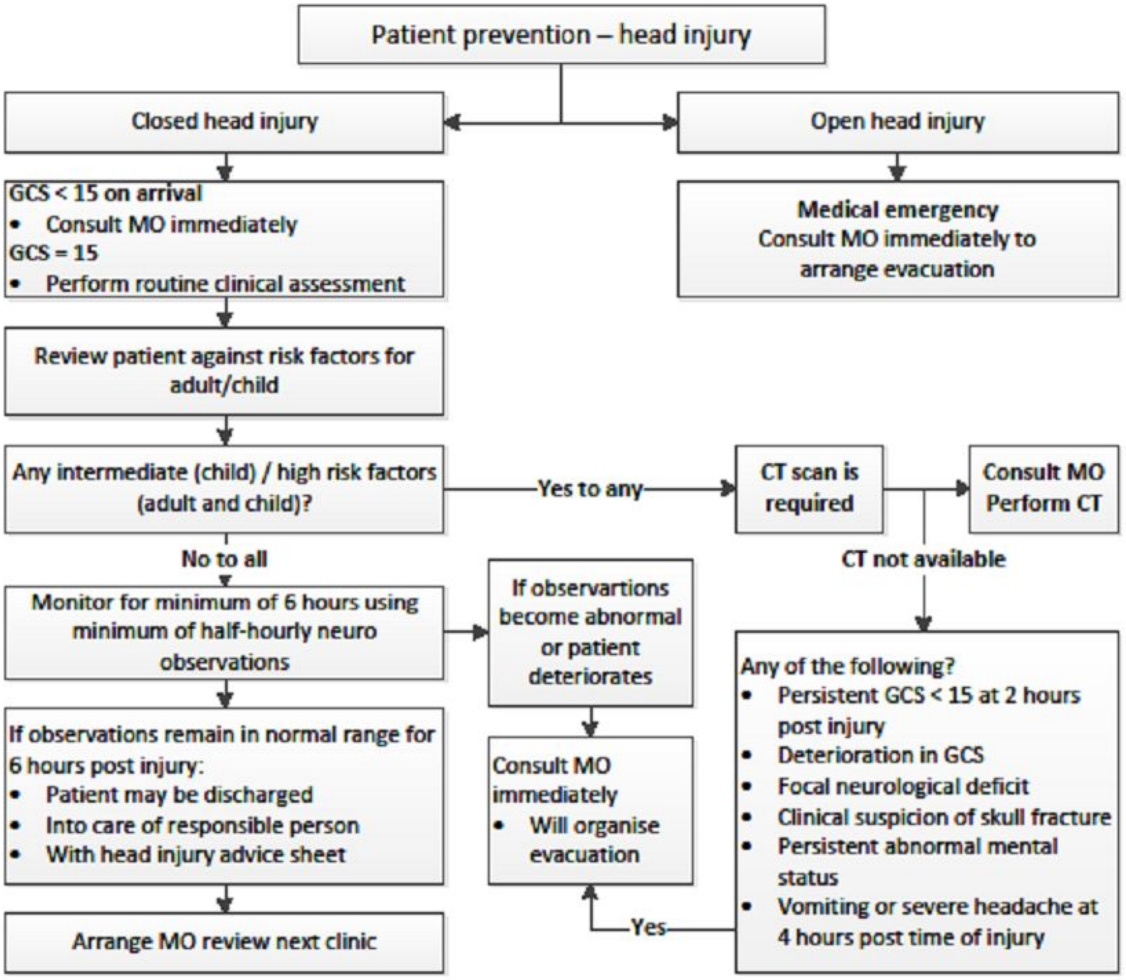
Part A: Texts A - D

Text A

Primary Clinical Care Guidelines: Management of Head Injuries

- Monitor observations including BP (blood pressure) and GCS (level of consciousness according to the Glasgow Coma Scale 1 – 15) .
- Notify MO (Medical Officer) immediately if level of consciousness alters.
- Prepare for intubation if GCS is 8 or less.
- Maintain BP as advised by MO.
- Keep patient warm.
- If there is a rapid deterioration in GCS of 2 or more and/or if one pupil becomes fixed and dilated, this may indicate expanding (intracranial) haemorrhage. Consult MO immediately.
- Give opioids with caution to patients with head injuries.
- If the skin is broken, check tetanus vaccination status. Administer tetanus containing vaccine/ immunoglobulin as appropriate.
- Assume all head injuries have an associated neck injury.

Text B


Patient prevention – head injury
Closed head injury
Open head injury
GCS < 15 on arrival
• Consult MO immediately
GCS = 15
• Perform routine clinical assessment
Medical emergency
Consult MO immediately to arrange evacuation
Review patient against risk factors for adult/child
Any intermediate (child) / high risk factors (adult and child)?
Yes to any
CT scan is required
Consult MO
Perform CT
No to all
CT not available
Monitor for minimum of 6 hours using minimum of half-hourly neuro observations
If observartions become abnormal or patient deteriorates
Any of the following?
• Persistent GCS < 15 at 2 hours post injury
• Deterioration in GCS
• Focal neurological deficit
• Clinical suspicion of skull fracture
• Persistent abnormal mental status
• Vomiting or severe headache at 4 hours post time of injury
If observations remain in normal range for 6 hours post injury:
• Patient may be discharged
• Into care of responsible person
• With head injury advice sheet
Consult MO immediately
• Will organise evacuation
Yes
Arrange MO review next clinic

Text C

Head injury clinical features – child	Intermediate risk factors	High risk factors
Age	< 1 year	
Witnessed loss of consciousness	< 5 minutes	> 5 minutes
Anterograde or retrograde amnesia	Possible	> 5 minutes
Behaviour	Mild agitation or altered behaviour	Abnormal drowsiness
Episodes of vomiting without other cause	3 or more	
Seizure in non-epileptic patient	Impact only	Yes
Non-accidental injury is suspected/parental history is inconsistent with injury	No	Yes
History of coagulopathy, bleeding disorder or previous intracranial surgery	No	Yes
Comorbidities	Present	Present
Headache	Yes	Persistent or increasing
Motor vehicle accident	< 60 kph	> 60 kph
Fall	1-3 metres	> 3 metres
Force	Moderate impact or unclear mechanism	High speed / heavy projectile or object
Glasgow Coma Scale	14-15	< 14
Focal neurological abnormality	Nil	Present
Injury	Haematoma, swelling or laceration > 5 cm	Penetrating injury / Possible depressed skull fracture.

Text D

Advice for patients who have received an injury to the head

- Rest quietly for the day.
- Use 'ice packs' over swollen or painful areas. Wrap ice cubes, frozen peas or a sports ice pack in a towel. Do not put ice directly on the skin.
- Take simple painkillers for any headache.
- If an injured patient is discharged in the evening, make sure they are woken several times during the night.
- Do not let the injured patient drive home.
- Do not leave them alone for the next 24 hours.
- Do not let them drink alcohol for at least 24 hours.
- Do not let them eat or drink for the first six to 12 hours (unless advised otherwise by the MO). Then offer them food and drink in moderation.
- Do not let them take sedatives or other medication unless instructed.
- Return to the clinic immediately if the patient has repeated vomiting, 'blacks out', has a seizure/fit or cannot be woken or is not responsive.
- Patient to return to clinic if they have any symptoms they or the carer are concerned about.

Part A

TIME: 15 minutes

- Look at the four texts, A-D, in the separate Text Booklet.
- For each question, 1-20, look through the texts, A-D, to find the relevant information.
- Write your answers on the spaces provided in this Question Paper.
- Answer all the questions within the 15-minute time limit.
- Your answers should be correctly spelt.

Head injuries: Questions

Questions 1-5

For each question, **1-5**, decide which text (**A**, **B**, **C** or **D**) the information comes from. You may use any letter more than once.

In which text can you find information about

1 what patients should and shouldn't do when they return home? _____

2 the possible cause of abnormality apparent in a patient's eyes? _____

3 reasons why patients should seek medical attention after being discharged? _____

4 procedures to follow dependent on the type of head injury? _____

5 past interventions and conditions to be considered when assessing risk? _____

Questions 6 – 11

Answer each of the questions, **6-11**, with a word or short phrase from one of the texts. Each answer may include words, numbers or both.

Children presenting with head injuries are assessed as high risk if they have:

had memory loss lasting **(6)** _____ or more

fallen **(7)** _____ or more

been hit by a weighty object or one moving at **(8)** _____

unusual levels of **(9)** _____

a **(10)** _____ which gets worse over time

Escalation: Children assessed as intermediate or high risk should undergo a **(11)** _____

Questions 12 – 16
Complete the sentences below by using a word or short phrase from the text. Each answer may include words, numbers or both.

All patients presenting with **(12)** _____ head injuries must be referred straight to the MO.

Patients with GCS below 8 may need **(13)** _____.

The MO should be informed without delay if there is a drop in BP or change in a patient's level of **(14)** _____.

Staff should be especially careful when administering **(15)** _____ to head injury patients.

Head injury patients may also have an injury to their **(16)** _____.

Questions 17 – 20

Answer the questions below. For each answer use a word or short phrase from the text. Each answer may include words, numbers or both.

17 If there are no significant risk factors, how long after a head injury can you discharge a patient?

_____

18 What should you provide head injury patients with when you discharge them?

_____

19 What should you advise patients to take to control headaches?

_____

20 What can patients use to avoid contact between ice packs and their skin?

_____